BASELINE → TREATMENT → FOLLOW-UP

When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm that either has happened.

Check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Blood / kidney / liver	full blood count (FBC), renal and liver profile	Confirm treatment-day results	Repeat on the source/local schedule	Review unresolved abnormalities	Individual clinical plan
Heart / breathing	Baseline cardiac assessment	Start 12-week monitoring clock	Echo/multigated acquisition (heart pumping) scan (MUGA) every 12 weeks	Keep clock through phase change	Product-information / risk-based plan
Cancer review	Confirm stage and setting	Agree review checkpoints	Review symptoms and response	Discuss continuation / next treatment	Breast follow-up
Other checks	Confirm HER2 and baseline eligibility	Review support medicines	Review toxicity before each dose	Endocrine / bone plan where relevant	Follow unresolved symptoms

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. National Cancer Control Programme (NCCP) 00815 v3 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up · EMA Herceptin product information · formulation and cardiac surveillance

THE WHOLE TREATMENT JOURNEY

Weekly paclitaxel + trastuzumab → trastuzumab continuation: treatment and checks

Adjuvant HER2-positive, node-negative breast cancer; tumour ≤3 cm

Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate. The team selects intravenous (IV) trastuzumab or locally approved subcutaneous trastuzumab; these are alternative formulations, not additional medicines.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment block	During treatment	Change / completion	Follow-up
Cancer medicines NCCP 00815	Confirm exact regimen	Weekly paclitaxel + trastuzumab → trastuzumab continuation	Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate. The team selects IV trastuzumab or locally approved subcutaneous trastuzumab; these are alternative formulations, not additional medicines.	Review active medicines and purpose	Next treatment / follow-up plan
Surgery / radiotherapy NCCP 00815	Adjuvant HER2-positive, node-negative breast cancer; tumour ≤3 cm	Follow the multidisciplinary team (MDT) sequence	Radiotherapy timing needs agreement	Selected local treatment as indicated	Review recovery and ongoing effects
Blood / kidney / liver NCCP 00815	FBC, renal and liver profile	Confirm treatment-day results	Repeat on the source/local schedule	Review unresolved abnormalities	Individual clinical plan
Heart / breathing NCCP 00815 · SMPC	Baseline cardiac assessment	Start 12-week monitoring clock	Echo/MUGA every 12 weeks	Keep clock through phase change	Product-information / risk-based plan
Cancer review NCCP 00815 · EARLY	Confirm stage and setting	Agree review checkpoints	Review symptoms and response	Discuss continuation / next treatment	Breast follow-up
Other checks NCCP 00815 · BONE	Confirm HER2 and baseline eligibility	Review support medicines	Review toxicity before each dose	Endocrine / bone plan where relevant	Follow unresolved symptoms

What each check is for

1 Baseline review

Before treatment

Adjuvant HER2-positive, node-negative breast cancer; tumour ≤3 cm. Assess cardiac function before trastuzumab and every 12 weeks during treatment. Keep the same monitoring clock when paclitaxel ends or trastuzumab changes schedule; arrange earlier review for symptoms or falling function. EMA product information recommends cardiac checks every 6 months after treatment until 24 months; following anthracyclines in early disease, yearly up to 5 years from the last trastuzumab dose or longer if function falls. Your team individualises this plan.

Ask / **act:** Confirm source eligibility and results.

Sources: NCCP 00815 · SMPC

2 Treatment-day check

Before each planned dose

Baseline blood tests and review before each chemotherapy administration according to the source and local plan. Once chemotherapy ends, testing follows trastuzumab and individual clinical needs.

Ask / **act:** Confirm active medicines and whether a dose is held.

Sources: NCCP 00815

3 Support plan

At every phase or day change

Paclitaxel weekly for 12 doses with trastuzumab every 3 weeks; continue trastuzumab to complete one year of HER2 treatment when appropriate. The team selects IV trastuzumab or locally approved subcutaneous trastuzumab; these are alternative formulations, not additional medicines.

Ask / **act:** Choose the matching support script; do not merge phases.

Sources: NCCP 00815

4 Response and follow-up

Agreed review points

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine computed tomography (CT)/positron emission tomography (PET) scans or blood tumour markers are not used to look for distant recurrence in a person without symptoms or clinical concerns.

Ask / **act:** Review symptoms, results and patient priorities together.

Sources: NCCP 00815 · EARLY

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-10-25 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.18